

Important Information. Please read carefully.

PROVINACE® TABLETS



COMPOSITION- Each tablet contains Perindopril erbumine 4 mg.

PHARMACODYNAMICS- Perindopril is an inhibitor of the enzyme that converts angiotensin I into angiotensin II (Angiotensin Converting Enzyme ACE). The converting enzyme, or kinase, is an exopeptidase that allows conversion of angiotensin I into the vasoconstrictor angiotensin II as well as causing the degradation of the vasodilator bradykinin into an inactive heptapeptide. Inhibition of ACE results in a reduction of angiotensin II in the plasma, which leads to increased plasma renin activity (by inhibition of the negative feedback of renin release) and reduced secretion of aldosterone. Since ACE inactivates bradykinin, inhibition of ACE also results in an increased activity of circulating and local kallikrein-kinin systems (and thus also activation of the prostaglandin system). It is possible that this mechanism contributes to the blood pressure-lowering action of ACE inhibitors and is partially responsible for certain of their side effects (e.g. cough). Perindopril acts through its active metabolite, perindoprilat. The other metabolites show no inhibition of ACE activity in vitro.

Hypertension- Perindopril is active in all grades of hypertension: mild, moderate, severe; a reduction in systolic and diastolic blood pressures in both supine and standing positions is observed. Perindopril reduces peripheral vascular resistance, leading to blood pressure reduction. As a consequence, peripheral blood flow increases, with no effect on heart rate. Renal blood flow increases as a rule, while the glomerular filtration rate (GFR) is usually unchanged. The antihypertensive activity is maximal between 4 and 6 hours after a single dose and is sustained for at least 24 hours: trough effects are about 87-100 % of peak effects. The decrease in blood pressure occurs rapidly. In responding patients, normalisation is achieved within a month and persists without the occurrence of tachyphylaxis. Discontinuation of treatment does not lead to a rebound effect. Perindopril reduces left ventricular hypertrophy. In man, perindopril has been confirmed to demonstrate vasodilatory properties. It improves large artery elasticity and decreases the media: lumen ratio of small arteries. An adjunctive therapy with a thiazide diuretic produces an additive-type of synergy. The combination of an ACE inhibitor and a thiazide also decreases the risk of hypokalaemia induced by the diuretic treatment.

Heart failure- Perindopril reduces cardiac work by a decrease in pre-load and after-load. Studies in patients with heart failure have demonstrated:

- decreased left and right ventricular filling pressures.
- reduced total peripheral vascular resistance.
- increased cardiac output and improved cardiac index.

PHARMACOKINETICS- After oral administration, the absorption of perindopril is rapid and the peak concentration complete within 1 hour. Bioavailability is 65 to 70 %. About 20 % of the total quantity of perindopril absorbed is converted into perindoprilat, the active metabolite. In addition to active perindoprilat, perindopril yields five metabolites, all inactive. The plasma half-life of perindopril is equal to 1 hour. The peak plasma concentration of perindoprilat is achieved within 3 to 4 hours. As ingestion of food decreases conversion to perindoprilat, hence bioavailability, PROVINACE should be administered orally in a single daily dose in the morning before a meal. The volume of distribution is approximately 0.2 L/kg for unbound perindoprilat. Protein binding is slight (binding of perindoprilat to angiotensin converting enzyme is less than 30 %), but is concentration-dependent. Perindoprilat is eliminated in the urine and the half-life of the unbound fraction is approximately 3 to 5 hours. Dissociation of perindoprilat bound to angiotensin converting enzyme leads to an "effective" elimination half-life of 25 hours, resulting in steady-state within 4 days. After repeated administration, no accumulation of perindopril is observed. Elimination of perindoprilat is decreased in the elderly, and also in patients with heart or renal failure. Dosage adjustment in renal insufficiency is desirable depending on the degree of impairment (creatinine clearance). Dialysis clearance of perindoprilat is equal to 70 ml/min. Perindopril kinetics are modified in patients with cirrhosis: hepatic clearance of the parent molecule is reduced by half. However, the quantity of perindoprilat formed is not reduced and therefore no dosage adjustment is required (see also "Dosage and Administration" and "Warnings and Precautions").

INDICATIONS- Hypertension: Treatment of hypertension

Heart Failure: Treatment of symptomatic heart failure

Stable coronary artery disease : Reduction of risk of cardiac events in patients with a history of myocardial infarction and/or revascularisation.

DOSAGE AND ADMINISTRATION- To be administered orally. It is recommended that PROVINACE is taken once daily in the morning before a meal. The dose should be individualised according to the patient profile (see "Warnings and Precautions") and blood pressure response.

Hypertension- PROVINACE may be used in monotherapy or in combination with other classes of antihypertensive therapy. The recommended starting dose is 4 mg given once daily in the morning. Patients with a strongly activated renin-angiotensin-aldosterone system (in particular, renovascular hypertension, salt and/or volume depletion, cardiac decompensation or severe hypertension) may experience an excessive drop in blood pressure following the initial dose. A starting dose of 2 mg is recommended in such patients and the initiation of treatment should take place under medical supervision. The dose may be increased to 8 mg once daily after one month of treatment. Symptomatic hypotension may occur following initiation of therapy with PROVINACE; this is more likely in patients who are being treated concurrently with diuretics. Caution is therefore recommended since these patients may be volume and/or salt depleted. If possible, the diuretic should be discontinued 2 to 3 days before beginning therapy with PROVINACE (see "Warnings and Precautions"). In hypertensive patients in whom the diuretic cannot be discontinued, therapy with PROVINACE should be initiated with a 2 mg dose. Renal function and serum potassium should be monitored. The subsequent dosage of PROVINACE should be adjusted according to blood pressure response. If required, diuretic therapy may be resumed. In elderly patients treatment should be initiated at a dose of 2 mg which may be progressively increased to 4 mg after one month then to 8 mg if necessary depending on renal function (see table below).

Symptomatic heart failure- It is recommended that PROVINACE, generally associated with a non-potassium-sparing diuretic and/or digoxin and/or a beta blocker, be introduced under close medical supervision with a recommended starting dose of 2 mg taken in the morning. This dose may be increased by increments of 2 mg at intervals of no less than 2 weeks to 4 mg once daily if tolerated. The dose adjustment should be based on the clinical response of the individual patient.

In severe heart failure and in other patients considered to be at high risk (patients with impaired renal function and a tendency to have electrolyte disturbances, patients receiving simultaneous treatment with diuretics and/or treatment with vasodilating agents), treatment should be initiated under careful supervision (see "Warnings and Precautions"). Patients at high risk of symptomatic hypotension e.g. patients with salt depletion with or without hyponatraemia, patients with hypovolaemia or patients who have been receiving vigorous diuretic therapy should have these

conditions corrected, if possible, prior to therapy with PROVINACE.

Blood pressure, renal function and serum potassium should be monitored closely, both before and during treatment with PROVINACE (see "Warnings and Precautions").

Stable coronary artery disease-

PROVINACE should be introduced at a dose of 4mg once daily for two weeks, then increased to 8mg once daily, depending on renal function and provided that the 4mg dose is well tolerated. Elderly patients should receive 2mg once daily for one week, then 4mg once daily the next week, before increasing the dose up to 8mg once daily depending on renal function (see Table 1 "Dosage adjustment in renal impairment"). The dose should be increased only if the previous lower dose is well tolerated.

Dosage adjustment in renal impairment- Dosage in patients with renal impairment should be based on creatinine clearance as outlined in table 1 below:

Table 1: dosage adjustment in renal impairment

Creatinine clearance (ml/min)	Recommended dose
$Cl_{CR} \geq 60$	4 mg per day
$30 < Cl_{CR} < 60$	2 mg per day
$15 < Cl_{CR} < 30$	2 mg every other day
Haemodialysed patients *, $Cl_{CR} < 15$	2 mg on the day of dialysis

* Dialysis clearance of perindoprilat is 70 ml/min. For patients on haemodialysis, the dose should be taken after dialysis.

Dosage adjustment in hepatic impairment- No dosage adjustment is necessary in patients with hepatic impairment (see "Warnings and Precautions" and "Pharmacokinetics")

Paediatric use- Efficacy and safety of use in children has not been established. Therefore, use in children is not recommended.

CONTRAINDICATIONS

- Hypersensitivity to perindopril, to any of the excipients or to any other ACE inhibitor;
- History of angioedema associated with previous ACE inhibitor therapy;
- Hereditary or idiopathic angioedema;
- Second and third trimesters of pregnancy (see "Pregnancy and lactation").
- The concomitant use of PROVINACE with aliskiren-containing products is contraindicated in patients with diabetes mellitus or renal impairment (GFR<60ml/min/1.73m²).

WARNINGS AND PRECAUTIONS

Stable coronary artery disease-

If an episode of unstable angina pectoris (major or not) occurs during the first month of perindopril treatment, a careful appraisal of the benefit/risk should be performed before treatment continuation.

Hypotension- ACE inhibitors may cause a fall in blood pressure. Symptomatic hypotension is seen rarely in uncomplicated hypertensive patients and is more likely to occur in patients who have been volume-depleted e.g. by diuretic therapy, dietary salt restriction, dialysis, diarrhoea or vomiting, or who have severe renin-dependent hypertension (see "Interaction with Other Medicaments" and "Side Effects"). In patients with symptomatic heart failure, with or without associated renal insufficiency, symptomatic hypotension has been observed. This is most likely to occur in those patients with more severe degrees of heart failure, as reflected by the use of high doses of loop diuretics, hyponatraemia or functional renal impairment. In patients at increased risk of symptomatic hypotension, initiation of therapy and dose adjustment should be closely monitored (see "Dosage and Administration" and "Side Effects"). Similar considerations apply to patients with ischaemic heart or cerebrovascular disease in whom an excessive fall in blood pressure could result in a myocardial infarction or cerebrovascular accident. If hypotension occurs, the patient should be placed in the supine position and, if necessary, should receive an intravenous infusion of normal saline. A transient hypotensive response is not a contraindication to further doses, which can be given usually without difficulty once the blood pressure has increased after volume expansion. In some patients with congestive heart failure who have normal or low blood pressure, additional lowering of systemic blood pressure may occur with PROVINACE. This effect is anticipated and is usually not a reason to discontinue treatment. If hypotension becomes symptomatic, a reduction of dose or discontinuation of PROVINACE may be necessary.

Aortic and mitral valve stenosis / hypertrophic cardiomyopathy. As with other ACE inhibitors, PROVINACE should be given with caution to patients with mitral valve stenosis and obstruction in the outflow of the left ventricle such as aortic stenosis or hypertrophic cardiomyopathy.

Renal impairment- In cases of renal impairment (creatinine clearance < 60 ml/min) the initial perindopril dosage should be adjusted according to the patient's creatinine clearance (see "Dosage and Administration") and then as a function of the patient's response to treatment. Routine monitoring of potassium and creatinine are part of normal medical practice for these patients (see "Side Effects"). In patients with symptomatic heart failure, hypotension following the initiation of therapy with ACE inhibitors may lead to some further impairment in renal function. Acute renal failure, usually reversible, has been reported in this situation. In some patients with bilateral renal artery stenosis or stenosis of the artery to a solitary kidney, who have been treated with ACE inhibitors, increases in blood urea and serum creatinine, usually reversible upon discontinuation of therapy, have been seen. This is especially likely in patients with renal insufficiency. If renovascular hypertension is also present there is an increased risk of severe hypotension and renal insufficiency. In these patients, treatment should be started under close medical supervision with low doses and careful dose titration. Since treatment with diuretics may be a contributory factor to the above, they should be discontinued and renal function should be monitored during the first weeks of perindopril therapy. Some hypertensive patients with no apparent pre-existing renal vascular disease have developed increases in blood urea and serum creatinine, usually minor and transient, especially when perindopril has been given concomitantly with a diuretic. This is more likely to occur in patients with pre-existing renal impairment. Dosage reduction and/or discontinuation of the diuretic and/or PROVINACE may be required.

Haemodialysis patients- Anaphylactoid reactions have been reported in patients dialysed with high flux membranes, and treated concomitantly with an ACE inhibitor. In these patients consideration should be given to using a different type of dialysis membrane or different class of antihypertensive agent.

Kidney transplantation- There is no experience regarding the administration of perindopril in patients with recent kidney transplantation.

Hypersensitivity/Angioedema- Angioedema of the face, extremities, lips, mucous membranes, tongue, glottis and/or larynx has been reported rarely in patients treated with ACE inhibitors, including perindopril (see "Side Effects"). This may occur at any time during therapy. In such cases, perindopril should promptly be discontinued and appropriate monitoring should be initiated and continued until complete resolution of symptoms has occurred. In those instances where swelling was confined to the face and lips the condition generally resolved without treatment, although

antihistamines have been useful in relieving symptoms. Angioedema associated with laryngeal oedema may be fatal. Where there is involvement of the tongue, glottis or larynx, likely to cause airway obstruction, emergency therapy should be administered promptly. This may include the administration of adrenaline and/or the maintenance of a patent airway. The patient should be under close medical supervision until complete and sustained resolution of symptoms has occurred. Angiotensin converting enzyme inhibitors cause a higher rate of angioedema in black patients than in non-black patients. Patients with a history of angioedema unrelated to ACE inhibitor therapy may be at increased risk of angioedema while receiving an ACE inhibitor (See "Contraindications").

Anaphylactoid reactions during low-density lipoproteins (LDL) apheresis: Rarely, patients receiving ACE inhibitors during low-density lipoprotein (LDL) apheresis with dextran sulphate have experienced life-threatening anaphylactoid reactions. These reactions were avoided by temporarily withholding ACE inhibitor therapy prior to each apheresis.

Anaphylactic reactions during desensitization: Patients receiving ACE inhibitors during desensitisation treatment (e.g. hymenoptera venom) have experienced anaphylactoid reactions. In the same patients, these reactions have been avoided when the ACE inhibitors were temporarily withheld, but they reappeared upon inadvertent rechallenge.

Hepatic failure: Rarely, ACE inhibitors have been associated with a syndrome that starts with cholestatic jaundice and progresses to fulminant hepatic necrosis and (sometimes) death. The mechanism of this syndrome is not understood. Patients receiving ACE inhibitors who develop jaundice or marked elevations of hepatic enzymes should discontinue the ACE inhibitor and receive appropriate medical follow-up (see "Side Effects").

Neutropenia/Agranulocytosis/Thrombocytopenia/Anaemia: Neutropenia/agranulocytosis, thrombocytopenia and anaemia have been reported in patients receiving ACE inhibitors. In patients with normal renal function and no other complicating factors, neutropenia occurs rarely. Perindopril should be used with extreme caution in patients with collagen vascular disease, immunosuppressant therapy, treatment with allopurinol or procainamide, or a combination of these complicating factors, especially if there is pre-existing impaired renal function. Some of these patients developed serious infections, which in a few instances did not respond to intensive antibiotic therapy. If perindopril is used in such patients, periodic monitoring of white blood cell counts is advised and patients should be instructed to report any sign of infection.

Race: ACE inhibitors cause a higher rate of angioedema in black patients than in non-black patients. As with other ACE inhibitors, perindopril may be less effective in lowering blood pressure in black people than in non-blacks, possibly because of a higher prevalence of low-renin states in the black hypertensive population.

Cough: Cough has been reported with the use of ACE inhibitors. Characteristically, the cough is non-productive, persistent and resolves after discontinuation of therapy. ACE inhibitor-induced cough should be considered as part of the differential diagnosis of cough.

Surgery/Anaesthesia: In patients undergoing major surgery or during anaesthesia with agents that produce hypotension, perindopril may block angiotensin II formation secondary to compensatory renin release. The treatment should be discontinued one day prior to the surgery. If hypotension occurs and is considered to be due to this mechanism, it can be corrected by volume expansion.

Hyperkalaemia: Elevations in serum potassium have been observed in some patients treated with ACE inhibitors, including perindopril. Patients at risk for the development of hyperkalaemia include those with renal insufficiency, uncontrolled diabetes mellitus, or those using concomitant potassium-sparing diuretics, potassium supplements or potassium-containing salt substitutes; or those patients taking other drugs associated with increases in serum potassium (e.g. heparin). If concomitant use of the above-mentioned agents is deemed appropriate, regular monitoring of serum potassium is recommended.

Diabetic patients: In diabetic patients treated with oral antidiabetic agents or insulin, glycaemic control should be closely monitored during the first month of treatment with an ACE inhibitor. (See "Interaction with Other Medicaments. Anti diabetic").

Lithium: The combination of lithium and perindopril is generally not recommended (see "Interaction with Other Medicaments").

Potassium sparing diuretics, potassium supplements or potassium-containing salt substitutes: The combination of perindopril and potassium sparing diuretics, potassium supplements or potassium-containing salt substitutes is generally not recommended (see "Interaction with Other Medicaments").

Increased risk of birth defects, fetal and neonatal morbidity and death when used throughout pregnancy.

Dual blockade of the renin-angiotensin-aldosterone system (RAAS): There is evidence that the concomitant use of ACE-inhibitors, angiotensin II receptor blockers or aliskiren increases the risk of hypotension, hyperkalaemia and decreased renal function (including acute renal failure).

Dual blockade of RAAS through the combined use of ACE-inhibitors, angiotensin II receptor blockers or aliskiren is therefore not recommended. If dual blockade therapy is considered absolutely necessary, this should only occur under specialist supervision and subject to frequent close monitoring of renal function, electrolytes and blood pressure. ACE-inhibitors and angiotensin II receptor blockers should not be used concomitantly in patients with diabetic nephropathy.

INTERACTIONS WITH OTHER MEDICAMENTS

Diuretics: Patients on diuretics, and especially those who are volume and/or salt depleted, may experience excessive reduction in blood pressure after initiation of therapy with an ACE inhibitor. The possibility of hypotensive effects can be reduced by discontinuation of the diuretic, by increasing volume or salt intake prior to initiating therapy with low and progressive doses of perindopril.

Potassium sparing diuretics, potassium supplements or potassium-containing salt substitutes: Although serum potassium usually remains within normal limits, hyperkalaemia may occur in some patients treated with perindopril. Potassium sparing diuretics (e.g. spironolactone, triamterene, or amiloride), potassium supplements, or potassium-containing salt substitutes may lead to significant increases in serum potassium. Therefore the combination of perindopril with the above-mentioned drugs is not recommended (see "Warnings and Precautions"). If concomitant use is indicated because of demonstrated hypokalaemia they should be used with caution and with frequent monitoring of serum potassium.

Lithium: Reversible increases in serum lithium concentrations and toxicity have been reported during concomitant administration of lithium with ACE inhibitors. Concomitant use of thiazide diuretics may increase the risk of lithium toxicity and enhance the already increased risk of lithium toxicity with ACE inhibitors. Use of perindopril with lithium is not recommended, but if the combination proves necessary, careful monitoring of serum lithium levels should be performed (see "Warnings and Precautions").

Non-steroidal anti-inflammatory drugs (NSAIDs) including aspirin $\geq$ 3 g/day: The administration of a non-steroidal anti-inflammatory drug may reduce the antihypertensive effect of ACE inhibitors. Additionally, NSAIDs and ACE inhibitors exert an additive effect on the increase in serum potassium and may result in a deterioration of renal function. These effects are usually reversible. Rarely, acute renal failure may occur, especially in patients with compromised renal function such as those who are elderly or dehydrated.

Antihypertensive agents and vasodilators: Concomitant use of these agents may increase the hypotensive effects of perindopril. Concomitant use with nitroglycerin and other nitrates, or other vasodilators, may further reduce blood pressure.

Antidiabetic agents: Epidemiological studies have suggested that concomitant administration of ACE inhibitors and antidiabetic medicines (insulins, oral hypoglycaemic agents) may cause an increased blood-glucose lowering effect with risk of hypoglycaemia. This phenomenon appeared to be more likely to occur during the first weeks of combined treatment and in patients with renal impairment.

Acetylsalicylic acid, thrombolytics, beta-blockers, nitrates: Perindopril may be used concomitantly with acetylsalicylic acid (when used as a thrombolytic), thrombolytics, beta-blockers and/or nitrates.

Tricyclic antidepressants/Antipsychotics/Anaesthetics: Concomitant use of certain anaesthetic medicinal products, tricyclic antidepressants and antipsychotics with ACE inhibitors may result in further reduction of blood pressure (see "Warnings and Precautions").

Sympathomimetics: Sympathomimetics may reduce the antihypertensive effects of ACE inhibitors.

Clinical trial data has shown that dual blockade of the RAAS through the combined use of ACE-inhibitors, angiotensin II receptor blockers or aliskiren is associated with a higher frequency of adverse events such as hypotension, hyperkalaemia and decreased renal function (including acute renal failure) compared to the use of a single RAAS-acting agent.

PREGNANCY AND LACTATION

Pregnancy: ACE inhibitors have been shown to be strongly fetotoxic in animal studies. Recently available data indicate that ACE inhibitors can cause foetal and neonatal morbidity and mortality when administered to pregnant women.

PROVINACE should not be used during the first trimester of pregnancy. When a pregnancy is planned or confirmed, the switch to an alternative treatment should be initiated as soon as possible. Controlled studies with ACE inhibitors have not been done in humans, but in a limited number of cases with first trimester exposure there do not appear to have been any malformations consistent with human fetotoxicity as described below. PROVINACE is contraindicated during the second and third trimesters of pregnancy. Prolonged ACE inhibitor exposure during the second and third trimesters is known to induce human fetotoxicity (decreased renal function, oligohydramnios, skull ossification retardation) and neonatal toxicity (renal failure, hypotension, hyperkalaemia). Such exposure to perindopril have occurred from the second trimester of pregnancy, ultrasound check of renal function and skull is recommended.

Lactation: It is not known whether perindopril is excreted into human breast milk. Therefore the use of PROVINACE is not recommended in women who are breast-feeding.

SIDE EFFECTS- The following undesirable effects have been observed during treatment with perindopril and ranked under the following frequency: Very common ($>1/10$); common ($>1/100$, $<1/10$); uncommon ($>1/1000$, $<1/100$); rare ($>1/10000$, $<1/1000$); very rare ($<1/10000$). including isolated reports.

Psychiatric disorders: Uncommon: mood or sleep disturbances

Nervous system disorders: Common: headache, dizziness, vertigo, paresthesia. Very rare: confusion

Eye disorders: Common: vision disturbance

Ear and labyrinth disorders: Common: tinnitus

Cardio-vascular disorders: Common: hypotension and effects related to hypotension. Very rare: arrhythmia, angina pectoris, myocardial infarction and stroke, possibly secondary to excessive hypotension in high risk patients (see "Warnings and Precautions").

Respiratory, thoracic and mediastinal disorders: Common: cough, dyspnoea. Uncommon: bronchospasm. Very rare: eosinophilic pneumonia, rhinitis.

Gastro-intestinal disorders: Common: nausea, vomiting, abdominal pain, dysgeusia, dyspepsia, diarrhoea, constipation. Uncommon: dry mouth. Very rare: pancreatitis.

Hepato-biliary disorders: Very rare: hepatitis either cytolytic or cholestatic (see "Warnings and Precautions").

Skin and subcutaneous tissue disorders: Common: rash, pruritus. Uncommon: angioedema of face, extremities, lips, mucous membranes, tongue, glottis and/or larynx, urticaria (see "Warnings and Precautions"). Very rare: erythema multiforme.

Musculoskeletal, connective tissue and bone disorders: Common: muscle cramps.

Renal and urinary disorders: Uncommon: renal insufficiency. Very rare: acute renal failure.

Reproductive system and breast disorders: Uncommon: impotence.

General disorders: Common: asthenia. Uncommon: sweating.

Blood and the lymphatic system disorders: Decreases in haemoglobin and haematocrit, Thrombocytopenia, leucopenia/neutropenia, and cases of agranulocytosis or pancytopenia, have been reported very rarely. In patients with a congenital deficiency of G-6PDH, very rare cases of pancytopenia, have been reported very rarely. In patients with a congenital deficiency of G-6PDH, very rare cases of haemolytic anaemia have been reported (see "Warnings and Precautions").

Investigations: Increases in blood urea and plasma creatinine, hyperkalaemia reversible on discontinuation may occur, especially in the presence of renal insufficiency, severe heart failure and renovascular hypertension. Elevation of liver enzymes and serum bilirubin have been reported rarely.

SYMPTOMS AND TREATMENT OF OVERDOSAGE

Limited data are available for overdosage in humans. Symptoms associated with overdosage of ACE inhibitors may include hypotension, circulatory shock, electrolyte disturbances, renal failure, hyperventilation, tachycardia, palpitations, bradycardia, dizziness, anxiety, and cough. The recommended treatment of overdosage is intravenous infusion of normal saline solution. If hypotension occurs, the patient should be placed in the shock position. If available, treatment with angiotensin II infusion and/or intravenous catecholamines may also be considered. Perindopril may be removed from the general circulation by haemodialysis. (See "Warnings and Precautions. Haemodialysis Patients"). Pacemaker therapy is indicated for therapy-resistant bradycardia. Vital signs, serum electrolytes and creatinine concentrations should be monitored continuously.

SHELF LIFE: The expiry date is indicated on the packaging.

DESCRIPTION- White colour, normal convex tablet with break line on one side and plain on the reverse. 10 tablets per blister pack in packs of 30's and 100's.

STORAGE- Store below 30°C. Protect from moisture.

KEEP ALL MEDICINES OUT OF REACH OF CHILDREN/ JAUHI DARI KANAK-KANAK

For further information, please consult your pharmacist or physician.

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